

Interstitial Cystitis / Bladder Pain Syndrome

Patient Instructions · Ongoing bladder pain and urgency that is not an infection

WARWIKI

Interstitial cystitis, also called bladder pain syndrome (IC/BPS), is a long-term condition of **bladder or pelvic pain, pressure, or discomfort** along with a frequent or urgent need to urinate — **without** an infection causing it. The discomfort often builds as the bladder fills and eases after you urinate. It can be managed well, even though there is no one-step cure.

About This Condition

IC/BPS is not an infection and is not contagious. Symptoms vary from person to person and tend to come in **flares** (worse spells) and quieter periods. Common features:

- Pain, pressure, or discomfort in the bladder or pelvis
- A frequent and/or urgent need to urinate, day and night
- Discomfort that **worsens as the bladder fills** and improves after urinating
- Flares that may follow certain foods, stress, or your menstrual cycle

What Causes It

The exact cause is not known. It likely involves a combination of a sensitive or irritated bladder lining, over-sensitive nerves, and (often) a **tight, overactive pelvic floor**. Because it has several drivers, treatment usually combines a few approaches.

LEARN THE TERMS

IC/BPS

Interstitial cystitis / bladder pain syndrome — bladder pain and urgency, not from infection.

Flare

A spell of worse symptoms, often triggered by food or stress.

Diagnosis of exclusion

Diagnosed after other causes (like infection) are ruled out.

Pelvic-floor therapy

Physical therapy to release tight pelvic muscles.

Bladder instillation

A soothing medicine placed into the bladder by a thin catheter.

Hunner lesion

A specific bladder-wall patch seen in some people, treatable directly.

Cystoscopy

A look inside the bladder with a thin camera.

WILL IT EVER GO AWAY? IC/BPS is usually a long-term condition, but most people get real relief by combining treatments and learning their triggers. The goal is fewer and milder flares and a normal daily life — it often takes some trial and error to find your mix, so be patient with the process.

How It's Diagnosed

There is no single test. Your team makes the diagnosis by your symptoms after ruling out other causes:

- A **urine test** to rule out infection or blood
- A symptom history and **bladder diary**
- Sometimes a **cystoscopy** to look for a Hunner lesion

How It's Treated (Step by Step)

- 1 **Self-care** — learn your **food triggers**, manage stress and sleep, and use gentle bladder training.
- 2 **Pelvic-floor physical therapy** — to release tight muscles (note: forceful Kegels can make it worse).
- 3 **Medicines** — oral options, and soothing **bladder instillations**.
- 4 **Targeted & advanced care** — treating a Hunner lesion, or nerve stimulation in stubborn cases.

Living With IC/BPS

- Common triggers include **coffee, tea, alcohol, citrus, tomatoes, and spicy foods** — try removing, then adding back to learn yours.
- Have a **flare plan** (rest, heat, extra water, soothing foods).
- Stress and a tight pelvic floor feed the cycle — gentle movement and relaxation help.

Call your team if you have:

- A fever, chills, or back/flank pain (possible infection)
- **Blood in the urine** that is new
- A flare you cannot control, or you cannot urinate

THREE THINGS TO REMEMBER

1. IC/BPS is bladder/pelvic pain with urgency that is **not** an infection — long-term, but very manageable.
2. Treatment combines self-care and trigger control, pelvic-floor therapy, medicines and instillations, and targeted care for specific findings.
3. Learn your food and stress triggers and keep a flare plan. New blood in the urine, fever, or flank pain should be checked.